جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاطمه سليمان علي زايد
المحافظة : الجيزه **الحي / المدينة :** الصف **العنوان :** الصف
رقم البطاقة : 25801142101006 **رقم التليفون :** 01155950043 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 14-01-1958 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي الاورام المنتشر [14456] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاطمه سليمان علي زايد
National ID: 25801142101006

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pin Medication And Anti Emetics

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

[[DOCTYPE:MDT_FORM]]
MDT Form

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

10/12/2026

MDT FORM

Name:	فاطمه سليمان علي Zayed	**MRN:**	12882
Age:	69 Y	**ID:**	25801142101006
Menopausal Sta	postmenopausal	**Phone:***	01155446226
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	Sister breast cancer
Gender			
CO Morbidity:	HTN	**Reason Of MDT:**	Disease progression
Surgical History:*	Internal fixation of hip joint	**ECOG Performance Status**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4

Clinical Data:

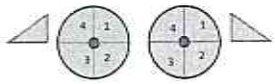
BX:

ER: **PR:** **HER2:** **KI67:**

History:

A CASE OF LT BREAST CANCER LOCALLY ADVANCED SINCE 10/2021 PT RECEIVED NA HORMONAL TTT AI SINCE 2021 AND WAS PLANNED TO UNDERWENT SURGERY BUT REFUSED PT CONTINUED ON HT AI TILL 2024 THEN PT HAS PATHOLOGICAL FRACTURE IN LT THIGH WITH FIXATION PT WAS NOT STICKY TO TTT LAST FU VISIT 8/2025 PT PRESENTED TODAY WITH BIL SM SHOWING PROGRESSIVE COURSE OF BILATERAL BREAST MASSES (BIRADS 6) LT SIDE MULTIPLE PATHOLOGICAL AXILLARY , INFRA AND SUPRACLAVICULAR AXILLARY LNS O/E : EXTENSIVE LT BREAST LESION WITH MUTIPLE SKIN NODULES

R L



T N M

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Sonomammography:

Clinical Staging:

Biomarker/Molecular

Pathological Staging:

Mets Workup:

Ct CAP 5/10/2026....multiple variable sized pulmonary nodules likely metastatic.

Bone scan pending.

Recommendations:

For MDT to start cth ASAP consider q 3wks due to difficult mobility

MDT Outcome & Plan

For:

1.Bone scan

2.Echo & labs

3.Bring pt for clinical assessment.

Start Taxol/Carbo weekly accordingly

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د شيماء لاشين	د امنيه عيسوي		

12/10/2026 12/10/2026

[[DOCTYPE:CT_SCAN]]
CT Scan

Medical No. : 10842
Patient's Name: Fatima Suleiman Ali Zayed
Date: 2026-10-05
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

جوستاف روسي انترناشونال مصر

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST INJECTION REVEALED:

A known case of left breast CA with very dense mammary parenchymal pattern of the breast and an enhancing neoplastic lesion seen in its UOQ ,with central area of breaking down and /or cystic degeneration,irregular outlines and measures about 41 x33 mm in dimensions. The overlying skin is thickened and measures about 16mm. Multiple variable sized pathologically-enlarged left axillary lymph nodes are also seen and globally measure about 71x52mm in dimensions.

1-CHEST:

- A subpleural pulmonary nodule is seen in the middle lobe and in the posterior segment of both lower pulmonary lobes. The former is the largest one and measures about 10 mm
- Ground-glass opacities/ linear atelectatic bands are seen scattering in both pulmonary lobes with bilateral increased broncho-vascular markings and basal bronchiectatic changes.
- No calcified pulmonary foci or cavitary lesions.
- No sizable abnormally-enhancing hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Increased size of the heart and mild atherosclerotic changes seen in its great vessels.

2-ABDOMEN AND PELVIS:

- Mildly increased size of the liver,measuring about 16.5 cm, showing diffuse low parenchymal attenuation. No sizable abnormally-enhancing hepatic focal lesions or dilated biliary radicles.

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- Patent PV.
- Relatively thickened left adrenal gland.
- Normal C.T appearance of the spleen,measuring about 9.2 cm, pancreas, functioning both kidneys(apart from relatively smaller left kidney), right suprarenal gland, aorta and IVC.
- No sizable abnormally-enhancing abdominal/pelvic lymph nodes seen.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- An IUCD is seen within the uterine cavity.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No ascites.
- **Spondylo-degenerative changes of the scanned spine with no definite osteolytic/osteosclerotic bony lesions seen.**

IMPRESSIONS:

A known case of left breast CA with an enhancing neoplastic lesion in its UOQ ,having the above-mentioned criteria and extensions.The overlying skin is thickened with multiple variable sized pathologically-enlarged left axillary lymph nodes and subpleural pulmonary nodules noted in the middle lobe and in the posterior segment of both lower pulmonary lobes and relatively thickened left adrenal gland,likely to be metastatic in nature.

- Ground-glass opacities/ linear atelectatic bands scattering in both pulmonary lobes with bilateral increased broncho-vascular markings and basal bronchiectatic changes,likely to be inflammatory reactions.
- Mild hepatomegaly with fatty infiltration and no focal lesions.
- For clinical check up.

Prof/Hanan El-Ahwal.

Dr.Hanan Elahwel

مبـــادرة رئيس الجمهورية
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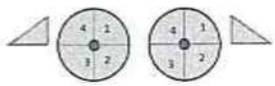
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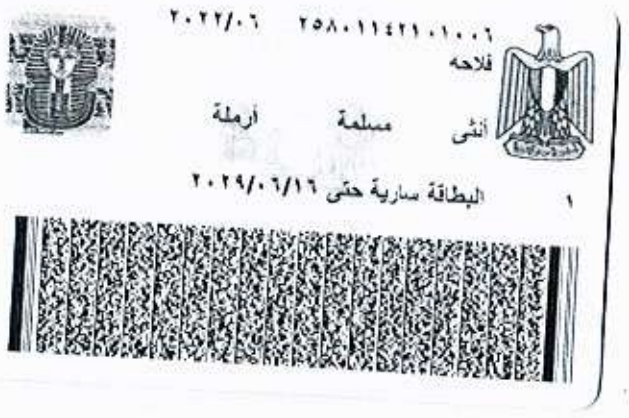
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MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د شيماء لاشين	د امنيه عيسوي		

12/10/2026 12/10/2026

MDT Form
[[DOCTYPE:MDT_FORM]]





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

National Women's Health Initiative
Pathology Lab
معمل باثولوجي المبادرة القومية لصحة المرأة

Pathology Report

Name: فاطمة سليمان على زايد	**Age: 63 years**
Clinical data: Left breast mass	
Specimen type: Tru cut biopsy	
Date (Received): 2 /8/2021.	**Date(reported): 15 /8/2021**
Lab. Number: 1650/ 21	**Hospital Number:** صحة مراه

Gross Examination:

Multiple grayish white tissue cores, longest 1.5 cm. Totally submitted.

Microscopic examination:

Histopathological examination of sections prepared from the Tru cut specimen received reveals an infiltrating malignant epithelial growth. The tumor is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II]. No intraduct component is detected . No lymphovascular invasion .

Diagnosis:

Left breast; Tru cut biopsy:

Invasive duct carcinoma, Grade II .

Biomarkers:

- **ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)**
- **PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)**
- **Her-2: Faint incomplete membrane reaction in < 10% of tumor cells , negative (score 1)**
- **Ki 67: 40 % nuclear reaction**

Prof. Dr. Lubna Omar

Dr. Samah Sayed

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